

SOUTH SUDAN HUMANITARIAN UPDATE



A boat with new arrivals (mostly mothers and children) docks at Dengjok Payam with IDP returnees from Tiergol in Ethiopia on 24 June 2026. Several people are still returning home to Akobo after fleeing clashes in March 2026.

Photo: OCHA/ Doreen Kansime

As of 26 June 2026

HIGHLIGHTS

- Latest seasonal forecasts indicate a high likelihood of below-average rainfall and above-average temperatures across much of South Sudan during the June–September rainy season.
- A substantial number of IDPs have reportedly returned to Akobo following an improvement in the security situation.
- An estimated 22,000 people in Udier Payam, Longochuk County, are in urgent need of humanitarian assistance, according to a recently concluded Inter-Agency Rapid Needs Assessment.
- The Emergency Relief Coordinator, Tom Fletcher, has announced an allocation of US\$6 million for Ebola operational readiness in South Sudan.

Climate outlook signals rising drought risks and food insecurity

On 25 June, the National Flood Taskforce reported that seasonal forecasts indicate a high likelihood of below-average rainfall and above-average temperatures across much of South Sudan during the June–September rainy season. River monitoring shows that water levels remain below seasonal norms in several locations, including Juba, Bor and Mangalla, suggesting a slower onset of flooding compared with recent years. However, forecasts point to elevated drought and water-stress risks, particularly in Eastern Equatoria, the Greater Pibor Administrative Area and northern Upper Nile, where some of the highest temperature anomalies are expected. Early impact is already emerging, with the Office of the United Nations High Commissioner for Refugees (UNHCR) reporting that about 1,500 people have moved from areas around Kapoeta in Eastern Equatoria towards Kenya due to water scarcity and deteriorating livelihoods. A significant number of women and children in Kapoeta reportedly intend to move to Kenya but lack the financial means, indicating that additional population movements may occur in the coming days. While localized flooding remains possible, the overall outlook indicates an increased risk of drought conditions, water shortages, livestock losses and reduced agricultural production, especially in the eastern and south-eastern areas. In response, the Flood Taskforce is updating national risk

scenarios and will finalize a revised preparedness and response plan covering both flood and drought contingencies. Meanwhile, the latest Food and Agriculture Organization of the United Nations (FAO) and the United Nations World Food Programme (WFP) [Hunger Hotspots report](#) highlights South Sudan as one of the four countries of highest global concern, driven by the scale and severity of acute food insecurity. [An estimated 7.8 million people are classified in IPC Phase 3 \(Crisis\)](#) or above during the lean season, according to the latest Integrated Food Security Phase Classification (IPC) analysis. The combined effects of conflict, climatic shocks, economic pressures and funding shortfalls are expected to worsen conditions through the second half of 2026. The projected climate outlook, alongside displacement, disease outbreaks and declining humanitarian funding, is likely to further exacerbate food insecurity and malnutrition, particularly in already vulnerable areas.

Jonglei State: significant return movements in Akobo East drive humanitarian needs

Return movements to Akobo East in Jonglei State have been substantial, with nearly the entire population reported to have returned. This includes more than 86,400 host community members, returnees and Internally Displaced Persons (IDPs) residing in Nyandit, Bilkey, Dengjok

and Gagdong payams. Of the 93,200 people targeted, WFP has verified over 75,000 people for food assistance across Bilkey, Nyandit, Dengjok and Gagdong payams. Latest seasonal forecasts indicate a high likelihood of below-average rainfall and above-average temperatures across much of South Sudan during the June– September rainy season.

As of 17 June, 45,200 people have been reached, with food distributions ongoing to cover the remaining verified beneficiaries and respond to evolving needs. In addition, about 11,200 IDPs from neighbouring Nyirol and Uror counties, as well as Akobo West, are currently hosted in Akobo East, including in Akobo Town, increasing pressure on limited resources and services. Needs have been assessed as high across sectors, with health, nutrition, water, sanitation and hygiene (WASH), protection and food consistently identified as top priorities. The nutrition status of children is particularly alarming. [Malnutrition screening](#) of 13,401 children of age 5 identified 2,308 with severe acute malnutrition (SAM) and 4,487 with moderate acute malnutrition (MAM). The United Nations Children's Fund (UNICEF) distributed ready-to-use therapeutic food to children with SAM during the assessment, and it remains a top priority in the response.

Inadequate safe water increases disease outbreak risk in Akobo County

The collapse of water and sanitation systems has created an extreme risk of disease outbreaks. Before the conflict, 17 water towers and 35 boreholes supplied Akobo through an underground network; these were destroyed or looted. As of 23 June, only 8 hand pumps remain functional, enough to serve about 5,000 people out of a population of over 100,000. Most people are still relying on untreated river water, and open defecation is widespread, increasing sanitation concerns. With malaria transmission already accelerating and the rainy season intensifying, there is extreme risk of waterborne disease outbreaks, including cholera, which has been spreading in Jonglei State since February. As of 21 June, humanitarian partners restored water supply to Akobo Hospital, supplying about 90,000 litres of water daily to the hospital and 15 community water points serving an estimated 20,000 people in Akobo Town. In parallel, daily infection prevention and control and environmental activities have been conducted; these include drainage excavation to reduce standing water, grass cutting for vector control, and deep cleaning and disinfection of hospital wards to improve conditions for patients.

Authorities call for assistance to IDP returnees in Akobo West and Canal/Pigi County

Over 21,000 conflict-affected people, including IDPs, are in urgent need of humanitarian assistance, particularly shelter, non-food items (NFIs) and protection, according to

authorities in Walgak, Akobo West. Humanitarian partners reported that access to Akobo West (especially the opposition-controlled areas) remained limited due to the volatile security situation, depriving hundreds of thousands of conflict-affected people of humanitarian assistance. In Canal/Pigi County, authorities also called for humanitarian support for IDP returnees in Atar, Wunekirr, Wunlueth, Mat, Khorwach and Allele 3 payams; exact figures are still being verified. Despite requests for support, ongoing clashes between government and opposition forces have made the area difficult for humanitarians to access. The security situation remains tense, with clashes leading to further displacement and possible civilian casualties in the area.

Fighting displaces civilians in Pochalla County, Greater Pibor Administrative Area

On 18 June, armed youth attacked a South Sudan People's Defence Forces (SSPDF) position in Raat, Pochalla Payam, triggering clashes. The incident displaced hundreds of civilians, many of whom fled towards the border with Ethiopia and surrounding areas. The attack is likely to exacerbate tensions and insecurity in Raat town, a disputed area affected by ongoing competition over administrative control and access to gold mining resources.



Mothers register for non-food items, including mosquito nets and soap, outside the Outpatient Therapeutic Feeding Programme (OTP) in Dengjok Payam, Akobo County on 24 June 2026.

Photo: ©OCHA/Doreen Kansime

Inter-agency assessment identifies critical humanitarian needs in Longochuk County, Upper Nile State

Preliminary findings from an Inter-Agency Rapid Needs Assessment (IRNA) conducted on 16 June indicate that an estimated 22,000 people in Udier Payam, Longochuk County, are in urgent need of humanitarian assistance. The most critical needs identified include food, WASH, nutrition, health and shelter. Nutrition screenings for children under five reveal high malnutrition levels, with a global acute malnutrition rate of above 40 per cent per cent, exceeding emergency thresholds. The nutrition facility in Udier is not functional due to a lack of supplies and access constraints. Community leaders reported that the last food distribution took place in December 2025. The health facility is also non-functional owing to shortages of medical supplies. There are no functional

boreholes or safe water sources, and the people are relying on unsafe water for domestic use. Many households lost shelters and non-food items (NFIs) during the conflict, with most people currently living in makeshift shelters. Women reported cases of gender-based violence (GBV), trauma and other protection concerns. However, there are no health or protection services available to support survivors. The Inter-Cluster Coordination Group (ICCG) in Malakal is mobilizing a response, contingent on access.

Continued influx from Sudan increases pressure in Renk County

Arrivals from Sudan into Renk County, Upper Nile State, continue, with over 4,400 new arrivals recorded between 1 and 16 June, mainly through Joda/Wunthou. Movements are driven by ongoing conflict in Sudan's Blue Nile State, with reports of increased casualties linked to intensified fighting. While no new war-wounded patients arrived between 12 and 19 June, the cumulative number stands at 195, with a small number still receiving care. The continued influx has placed additional strain on health services, prompting plans to scale up staffing at Renk Civil Hospital. Nutrition screenings for children under age 5 at Renk Transit Centre reveal high malnutrition levels, with a global acute malnutrition rate of 17.3 per cent, above emergency thresholds. Screening and treatment services continue, alongside targeted supplementary feeding for children and pregnant and lactating women. Congestion remains a concern at the Transit Centre, with the current population of about 6,000 as of mid May, which has exceeded its 3,000-person capacity. Authorities are expanding the Abu Khadra returnee site and have allocated land in Chemmedi Payam for refugee settlement to ease pressure and support onward relocation.

Government and UN reinforce commitment to durable solutions

On 16 June, the Government of South Sudan and the United Nations reaffirmed their commitment to advancing durable solutions during the second Steering Committee meeting in Juba, with a joint pledge to end protracted displacement for at least 60,000 people by the end of 2026. Progress under the 2024 National Durable Solutions Strategy was reviewed, highlighting that while returns and reintegration are advancing in some locations, outcomes remain dependent on access to basic services, land availability and local stability. State-level roadmaps have been rolled out in Upper Nile and Western Bahr el Ghazal, with plans underway for Unity State to strengthen coordinated humanitarian, development and peace interventions. To date, over 28,000 people have returned, reintegrated or resettled across the three states with support from the Government, United Nations agencies led by the International Organization for Migration (IOM), the United Nations Development Programme (UNDP) and UNHCR, and partners. This is alongside broader efforts supporting

livelihoods and housing, land and property rights.

Humanitarian access impediments disrupt aid operations

Humanitarian access remained constrained by violence against humanitarian personnel, assets and facilities, restriction of movement of agencies, personnel or goods, and interference in the implementation of humanitarian activities across the country. Humanitarian partners implementing multipurpose cash assistance are facing challenges with transporting cash from Juba to Akobo due to blockage by Military Intelligence at Juba International Airport. This blockage is affecting over 32,000 people registered to receive cash assistance in Akobo.

In Ulang County, Médecins Sans Frontières (MSF) operations were affected after the Ulang County Commissioner, in a letter dated 15 June to the State Governor, accused the organization of violating labour laws through recruitment practices, occupying a government school in Barmach without approval, and resuming activities in Barmach and Nyangore without informing county authorities. MSF on 15 June released a statement announcing the death of a staff member (a nurse) who worked at the Pieri Primary Health Care Centre (PHCC) project in Lankien. The staff member was confirmed killed on 28 May 2026 while travelling to Pieri. At least 24 aid workers, including contractors, have been killed since January. Humanitarian personnel, particularly national staff, continue to operate under extremely difficult conditions. Between January and May 2026, humanitarian partners reported 328 access incidents, representing a 74 per cent increase compared with the same period in 2025. Given persistent underreporting, the actual number is likely to be higher. The main constraints include conflict-related disruptions, violence against humanitarian personnel and assets, interference with humanitarian activities and bureaucratic impediments.

These challenges affect the speed, scale and cost of humanitarian operations, leaving vulnerable people exposed to greater risks. A humanitarian partner suspended operations in Mugwo Payam, Yei County, due to safety concerns after the abduction and detention of a female humanitarian worker, with perpetrators reportedly demanding a ransom of \$20,000. This incident, coupled with repeated abductions of aid personnel along the Yei–Morobo road, prompted Mugwo Development Organization (MDO) to suspend field missions on 11 June for monitoring food security and livelihoods activities and implementing capacity-building projects. In Malakal, Upper Nile State, authorities proposed new travel permit requirements for humanitarian missions, raising concerns over delays and restricted access to opposition-held areas with vulnerable populations. Access to Nasir, Ulang and nearby counties remains constrained by layered requirements, while ongoing interference continues to delay aid delivery.

Humanitarian access workshop held

A stakeholder engagement workshop on humanitarian access and protection of civilians was held in Bor on 23 and 24 June, calling for strengthened implementation of the 2017 Presidential Decree on free and unimpeded humanitarian access. The Governor of Jonglei State, Dr. Riek Gai Kok, acknowledged the critical role of humanitarian agencies in assisting vulnerable populations, while noting persistent operational challenges affecting the humanitarian response. OCHA facilitated sessions on humanitarian principles, humanitarian access and civilmilitary coordination. Participants included county commissioners, members of parliament, representatives of the Ministry of Foreign Affairs, the Jonglei Relief and Rehabilitation Commission (RRC), the Joint Border Verification and Monitoring Mechanism (JBVMM), SSPDF, and humanitarian partners.

Multiple public-health emergencies reported across South Sudan

South Sudan continues to face significant health needs driven by recurrent displacement, disease outbreaks, food insecurity and limited access to health care in hard-to-reach areas. Priority needs include uninterrupted access to essential primary health care, maternal and child health services, emergency obstetric care, nutrition services, disease surveillance, and adequate medical supplies. The Health Cluster partners continue to support the Ministry of Health in coordinating and responding to multiple public health emergencies, including cholera, polio, mpox, acute food insecurity and cross-border Ebola readiness.

Ebola readiness

South Sudan remains at high risk of Ebola virus disease (EVD) importation from neighbouring countries. Although no cases have been confirmed, the risk remains high due to the porous borders and a strained health-care system. The national overall readiness score improved from 29 per cent to 45 per cent as of 4 June. According to the World Health Organization (WHO), infection prevention and control stands at 8 per cent, contact tracing at 14 per cent and surveillance at 33 per cent. Preparedness measures have been strengthened since the outbreak was reported. At least 284,733 travellers have been screened at Points of Entry (P o E) as of 26 June, up from 223,179 as of 22 June. Preparedness measures expanded geographically with the establishment of local Incident Management System (IMS) structures and Rapid Response Teams in Nimule, enhanced surveillance and contact-tracing capacity, and the deployment of additional personal protective equipment (PPE) kits for viral haemorrhagic fever response. A recent simulation exercise confirmed that national coordination mechanisms are functional but highlighted persistent gaps, including limited state-level IMS structures, shortages of PPE and laboratory reagents

in some border areas, inadequate isolation infrastructure, and the absence of functional infectious waste disposal capacity. Key operational risks remain cross-border surveillance gaps with the Democratic Republic of the Congo (DRC), inconsistent pillar coordination meetings and unresolved waste management challenges. Immediate priorities include strengthening cross-border alert-sharing, operationalizing subnational response structures, accelerating approvals of standard operating procedures (SOPs) and improving supply chain coordination. The Ministry of Health has developed an updated EVD readiness and response plan for May to July, with an estimated cost of \$7.3 million. The plan will be implemented through a coordinated, multi-partner and multisectoral approach.

Cholera outbreak

The cholera outbreak remains active, with a total of 107,224 cases and 1,699 deaths (case fatality rate: 1.6 per cent) recorded across 55 counties in 9 states and 3 administrative areas as of 22 June. In the past week (15 to 21 June), 90 new cholera cases and no deaths were reported from 5 counties, a 34.3 per cent decrease from the previous week (137 cases). In Unity State, most cases were recorded at Bentiu IDP Camp, followed by Bentiu State Hospital and Yonyang. Children under age 5 accounted for 53 per cent of cases, with slightly more cases among females. Preparations for mop-up oral cholera vaccination (OCV) campaigns in Rubkona, Mayom and Leer counties of Unity State are underway, following the approval of 435,477 oral cholera vaccine doses by the International Coordinating Group on Vaccine Provision during the week ending 19 June.



A nutrition assistant at Medair's Outpatient Therapeutic Programme (OTP) site in Akobo Town provides mothers with nutrition education to support the recovery of children with acute malnutrition - 23 June 2026.

Photo: ©OCHA/Doreen Kansiime

CERF allocates \$6 million to strengthen Ebola preparedness amid

escalating humanitarian needs

Through the [Central Emergency Response Fund \(CERF\)](#), the [Emergency Relief Coordinator, Tom Fletcher](#), has allocated [\\$6 million for Ebola operational readiness in South Sudan](#). [South Sudan](#) faces a high risk of the spread of Ebola, given active population movements across its borders with both DRC and Uganda. The Government of South Sudan, through the Ministry of Health, has activated the national preparedness and response plan to strengthen readiness for a potential importation of Ebola in 15 priority counties, although additional funding is required to sustain these measures. This allocation will strengthen preparedness to rapidly detect, investigate and contain imported Ebola virus disease while protecting health workers, maintaining essential health.